

Skin Abnormalities in Patients with Gastrointestinal Bleeding

Congenital Malformations of Blood Vessels

- **Hereditary hemorrhagic telangiectasia**

Characterized by a familial pattern of epistaxis, mucosal telangiectases, and arteriovenous malformations in multiple organs. Gastrointestinal hemorrhage, typically from the stomach or duodenum, occurs in up to 40% of patients, with an average onset age of 55 years.

- **Blue rubber bleb nevus syndrome**

Most cases are sporadic, but autosomal dominant inheritance has been reported. Gastrointestinal venous malformations are commonly found in the small intestine, projecting into the lumen. These lesions bleed more readily than cutaneous ones, leading to chronic anemia or acute hemorrhage.

Inherited Defects of Connective Tissue

- **Ehlers-Danlos syndrome, vascular type (type IV)**

Autosomal dominant disorder associated with bowel rupture, particularly involving the sigmoid colon, which accounts for 25% of all complications and 8% of deaths. Diverticula and hernias may also occur.

- **Pseudoxanthoma elasticum**

Approximately 13% of patients experience gastrointestinal bleeding, usually from the stomach, often as the first clinical sign of the disease.

Other Dermatologic Conditions

- **Kaposi sarcoma**

The gastrointestinal tract, especially the small intestine, is a common site. While often asymptomatic, severe bleeding can occur.

- **Vasculitis**

May lead to gastrointestinal involvement and bleeding.

- **Polyposis syndromes**

Including Gardner syndrome and Peutz-Jeghers syndrome, associated with increased risk of gastrointestinal bleeding.

- **Ulcerative colitis and Crohn disease**

Inflammatory bowel diseases that can present with gastrointestinal bleeding.

- **Gastrointestinal tumors**

Primary or metastatic tumors may cause bleeding.

Drug-Induced Gastrointestinal Bleeding

- **Glucocorticoids**

Systemic use commonly causes dyspepsia; less frequently leads to gastric erosions, ulceration, and hemorrhage.

- **Methotrexate**

Can cause enteritis, gastrointestinal ulceration, and bleeding.

Skin Abnormalities in Patients with Abdominal Pain

Dermatologic Conditions with Abdominal Pain

- **Herpes zoster**

Involvement of sensory roots T6–L1 may cause abdominal pain before skin lesions appear. Sacral (S2–S4) involvement may lead to perineal pain and urinary or bowel dysfunction.

- **Angioedema**

Acute attacks of urticaria or angioedema may present as abdominal pain due to bowel wall edema and mucosal thickening, often with free peritoneal fluid visible on ultrasound. In hereditary angioedema, colicky abdominal pain and laryngeal edema require urgent treatment with purified C1 inhibitor concentrate.

- **Porphyria (variegate type)**

Rare form where skin lesions and abdominal pain coexist. Skin changes resemble those in porphyria cutanea tarda. Acute attacks may be triggered by pregnancy or drugs (e.g., estrogen, griseofulvin). Diagnosis is supported by detection of urinary uroporphyrin and fecal protoporphyrin, but plasma fluorescence at 626 nm is more sensitive, even during remission.

- **Fabry disease**

X-linked disorder that may present with abdominal pain and gastrointestinal symptoms.

- **Vasculitis**

Including Henoch-Schönlein purpura and other collagen vascular diseases, may cause abdominal pain due to gastrointestinal vasculitis.

- **Malignant atrophic papulosis (Degos disease)**

Characterized by skin lesions and gastrointestinal infarction leading to severe abdominal pain.

Polyposis Syndromes

- **Gardner syndrome**
- **Peutz-Jeghers syndrome**
- **Cronkhite-Canada syndrome**

All associated with gastrointestinal polyposis and risk of abdominal pain.

Neurocutaneous and Inflammatory Conditions

- **Neurofibromatosis**

May involve the gastrointestinal tract, causing abdominal pain.

- **Ulcerative colitis and Crohn disease**

Chronic inflammatory bowel diseases frequently associated with abdominal pain.

- **Pancreatitis**

Can be associated with skin conditions like hypertriglyceridemia or vasculitis.

Paraneoplastic Skin Syndromes

- **Dermatomyositis**
- **Acanthosis nigricans**
- **Hypertrichosis lanuginosa**

These may precede or accompany gastrointestinal malignancies and present with abdominal pain.

Note: Systemic mastocytosis involves the gastrointestinal tract in one-third of patients, potentially contributing to both abdominal pain and bleeding.